SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pertuzumab + trastuzumab · with selected chemotherapy

HER2-positive early breast cancer · neoadjuvant or adjuvant framework · NCCP 00350 · version 3 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Treat HER2-positive early breast cancer as part of a plan intended to reduce the risk of it returning.

Schedule
IV pertuzumab and IV trastuzumab every 21 days. Before surgery: 3–6 cycles alongside the selected chemotherapy. After surgery: up to 18 cycles overall under the agreed adjuvant plan. If an anthracycline is used first, finish it before starting these antibodies with the taxane phase. Confirm the exact companion regimen and local access.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pertuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction
- ☐ Treatment reaction
- ☐ Diarrhoea and rash
- ☐ Fatigue

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction
- ☐ Treatment reaction
- ☐ Diarrhoea and rash
- ☐ Fatigue

Discuss the treatment aim, alternatives, expected benefit, common and serious effects, fertility and pregnancy precautions, route, monitoring, practical support and the agreed contact plan. The selected chemotherapy needs its own information and consent discussion; this aid covers the antibody component only.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00350 v3.

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